

Management of STABLE angina

(NICE 2016, CG126)



Red Whale

GEMS

Guidelines & Evidence Made Simple

Management involves:

- **Lifestyle modification** (physical activity, stopping smoking, diet, weight control).
- **Drug therapy**, which has 3 strands
 - Short-acting nitrate for relief/prevention of symptoms (usually GTN).
 - Prevention of angina symptoms.
 - Secondary prevention of further cardiovascular episodes.
- **Revascularisation** (see main article for detail):
 - If symptoms are NOT controlled with drugs: consider revascularisation to improve symptoms.
 - If symptoms ARE controlled with drugs: revascularisation may be considered in some to improve prognosis.

DRUG THERAPY

SYMPTOM RELIEF

Short-acting nitrate for symptom relief/prevention (usually GTN)

- **Can be used prior to activity or for relief of pain.**
- Warn patients of side-effects: headache, flushing, lightheadedness (if affected, sit down/hold onto something).
- If using for pain relief, advise:
 - Stop activity and rest.
 - Use GTN.
 - Repeat dose after 5mins if the pain has not gone.
 - If pain has still not gone 5mins after second dose: call 999.

At Red Whale, we'd add:
ALWAYS CARRY GTN WITH YOU!

PREVENTION OF ANGINA

Titrate dose to control symptoms (up to max. dose).
Review 2–4w after any change

1ST-LINE AGENT: EITHER

Beta-blocker

or

calcium channel blocker (CCB)

Make choice based on comorbidities/contraindications/patient preference.

If one not tolerated, swap to the other one (if not contraindicated).

STEP UP IF INSUFFICIENT CONTROL ON 1ST-LINE AGENT

Swap to other 1ST-line drug

or

combine beta-blocker and CCB

(if combining CCB+B-blocker, must use dihydropyridine CCB (e.g. slow-release nifedipine, amlodipine or felodipine), not rate-limiting CCBs).

SECONDARY PREVENTION

of further cardiovascular events

Think: **SAB**

Statin: usually atorvastatin 80mg: NICE lipids guidance tells us **target is LDL $\leq 2\text{mmol/l}$ or non-HDL $\leq 2.6\text{mmol/l}$.**

- If target not met, consider additional agent (usually ezetimibe first).

- Even if cholesterol target met, can add ezetimibe to statin to further reduce risk. Details in lipids article.

Aspirin 75mg daily (taking into account comorbidities/risk of bleeding).

Blood pressure control: in line with NICE hypertension guidance.

If diabetes + stable angina, consider ACEi.

Why? Reduces CV events/deaths. (Use ACEi: absence of evidence for ARBs.)

WHEN TO ADD A THIRD DRUG?

Consider if:

- Symptomatic despite 2 drugs AND awaiting revascularisation OR revascularisation not appropriate/wanted.
- Do NOT add a third drug if symptoms are controlled on 2 agents.

What role for long-acting nitrates, nicorandil, ranolazine and ivabradine? Use if symptomatic and:

- Neither first-line therapy is tolerated.
- Symptomatic on single first-line agent and unable to take 2 first-line agents.
- If third agent indicated, in situations described above.

Base decision about which to use on comorbidities/contraindication/patient preference and drug costs: seek secondary care advice if unsure/unfamiliar with these drugs.

MHRA warnings about ivabradine (2014)

- Beware bradycardia: titrate up slowly as per BNF. Monitor for bradycardia/symptoms of bradycardia (e.g. dizziness, fatigue, hypotension). Reduce dose if experienced. Stop if resting heart rate remains below 60bpm/persisting symptoms.
- Do not exceed maintenance dose of 7.5mg twice daily.
- Do NOT use with rate-limiting CCB (e.g. verapamil, diltiazem).

MHRA warnings about nicorandil (2016)

Use nicorandil with caution. Do not use first line because of risk of serious ulcers in skin, eye and mucosa, including GI tract (with risk of fistulas). Risk greater if on NSAIDs or oral steroids, or if diverticular disease. Ulcers can first appear years after starting nicorandil. STOP if ulcers develop.

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. August 2024. For full references see the relevant Red Whale articles.